

TERE HEALTH AUSTRALIA

An invitation to co-found.

For Dr Patrick
Shively.

From

Dr Patrick Herling
Founder & CMO, Tere Health

Date

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Confidential

Tere is a working telemedicine platform.

Live in New Zealand today. Purpose-built for rural patients underserved by mainstream telehealth. Custom PMS — not a bolt-on to Medtech or Indici. Cash-pay plus ACC, no insurer credentialing.

We're building the Australian arm and we want you to co-found it with us — 33% equity, founding Medical Director for AU operations, working alongside two other founding partners.

Rural Australia is underserved and getting worse.

7M+

AUSTRALIANS LIVING REGIONAL OR REMOTE

40%

OF REMOTE COMMUNITIES WITHOUT A FULL-TIME
GP

\$500M

AUD CASH-PAY TELEHEALTH MARKET, GROWING
20%+ / YEAR

Existing players — Eucalyptus, Mosh, InstantConsult, Hola — are metro-focused and vertical (weight loss, men's health, sick notes). No dominant operator has positioned explicitly around rural / remote patients who can't see a GP today.

Not a slide deck. A shipped product.

Clinical core

- Video + phone consult with live AWS Transcribe subtitles
- AI-assisted note generation (AWS Bedrock, BAA-covered, Sydney region)
- E-prescribing under NZ Director-General signature exemption
- Structured allergies / medications / conditions
- AI triage with red-flag detection and manual clinician override

Infrastructure & compliance

- Provider MFA (TOTP), full audit logging, PHI-view justification prompts
- HIPC 2020 compliant, HDC-registered service, NEAC-mapped research
- ACC MST1 / MST3 claim lodgement, verified vendor
- Custom rPPG vitals (VitalsValidate) — proprietary IP
- Multi-language triage: English, Te Reo Māori, Samoan, Dutch, more

All shipped, all in production, all owned by Tere. Not licensed from a US or UK vendor.

The trans-Tasman lift is small. The market is big.

Regulatory tailwind

- MBA telehealth item numbers permanent post-COVID
- Trans-Tasman Mutual Recognition — NZ physicians register with AHPRA in weeks
- eScript infrastructure national (eRx / MediSecure)
- No CPOM-style corporate ownership restrictions

Technical tailwind

- All infrastructure already in AWS Sydney — zero migration
- Insurance uplift to include AU territory: ~\$3–5k AUD
- Entity + legal setup: ~\$10–20k AUD one-off
- English-language product with rural-fit positioning already validated in NZ

By contrast, US expansion is a 3–6 month regulatory lift with \$50k+ setup costs. AU is a 3–4 month lift with under \$20k in setup — with the same platform, same clinicians, and a market 5× the size of NZ.

Nobody is doing rural, generalist, credentialed, at speed.

PLAYER	FOCUS	MODEL	RURAL
Eucalyptus (Pilot, Juniper, Kin)	Vertical — weight loss, men's, women's	Cash subscription	No
Mosh	Men's health, hair loss, ED	Cash subscription	No
InstantConsult	Sick certs, private scripts	Cash per consult	Passively yes, not positioned
Hola Health / Doctors on Demand	General 24/7	Mixed MBS + private	No
MyEmergencyDr	Telehealth EM (closest peer)	Cash per consult	Passively yes, not positioned
Tere Health AU	Rural generalist urgent care	Cash per consult + eScript	Positioned explicitly

Rural doctors know the gap. Rural patients know the gap. Nobody has explicitly targeted them yet with a credentialed, generalist, video-first service.

Three founding partners. Complementary strengths.

Patrick Herling, D.O.

FOUNDER · CMO · TRANS-TASMAN

US ABEM board-certified emergency physician. MCNZ 99529. Six US state licences. Product leadership plus clinical governance. Built the Tere platform end-to-end.

Justin Thomas, MBA

FOUNDING PARTNER · HEAD OF OPERATIONS

Twenty years leading healthcare operations across NZ and the US. Currently Operations Manager, Radiology at Te Whatu Ora Marlborough. Prior Practice Manager at a cash-pay medical practice in Boise, ID. MBA in Healthcare Management. Blenheim-based — lives the rural gap Tere is being built to close.

Patrick Shively, MBBS

PROPOSED · AU MEDICAL DIRECTOR

Emergency medicine physician, 13 years across Australia and New Zealand — tertiary trauma centres through to remote / rural 2-bed EDs. Founder of Cosmabl (Sydney healthtech, AHPRA/TGA-compliant clinical governance platform). MBBS Wollongong; originally Colorado.

Founding Partner @ AU Medical Director.

You lead: Clinical governance for Australian operations. AU-licensed provider recruitment and supervision. AHPRA / MBA / RACGP / ACRRM representation. Sign-off on all AU clinical protocols.

You don't lead: Product engineering, platform infrastructure, NZ operations. Those stay with the existing team and Patrick Herling as CMO.

Time commitment

- 4–8 hours per week initially — negotiable
- Scales with AU consult volume
- Optional path to full-time as revenue supports it
- Represent Tere at 1–2 rural doctor conferences per year

You keep your practice

Continuing existing clinical work is expected. This is not an all-in ask.

AU clinical roster from Day 1. You and Dr Rachel Thomas (FACEM, registering with AHPRA via TTMRA ahead of launch) share the initial AU consult load. Patrick Herling leads platform and product engineering — the tech build never sits behind a regulatory queue. Justin Thomas runs operations for the AU practice. Clean division of labour: you and Rachel see patients, Patrick builds the platform, Justin runs the practice.

Equal partners in Tere Health Pty Ltd.

PATRICK HERLING · 33.3%

JUSTIN THOMAS · 33.3%

PATRICK SHIVELY · 33.3%

Corporate structure

- **Tere Health Pty Ltd** — new Australian entity for AU operations
- Three founding partners hold equal 33.3% shareholding
- Board: three directors, one seat each — equal voice, equal vote
- Independent of Tere Health Limited (NZ) at cap-table level; platform licensed in from NZ parent

A clean partnership between three founders — equal say, equal upside, no founder outranking another. Detailed shareholders agreement to be drafted together at term-sheet stage with independent legal review.

Cash-pay. Profitable early. No dilution planned.

Revenue model

- Cash-pay per consult, indicative \$80–120 AUD
- No insurance credentialing — no billing / coding overhead
- eScript e-prescribing (eRx or MediSecure)
- Direct-to-consumer via web + mobile
- Optional B2B: license Tere platform to other rural practices

Funding & unit economics

- Setup capital ~\$10–20k AUD (entity, insurance, integrations)
- Year 1 self-funded — no external investors
- Break-even at ~500 consults / month
- Profitability > growth in year 1
- Optional seed round at 18–24 months only if scale demands

All numbers indicative. Detailed AU-specific financial model to follow term-sheet stage.

At ten patients per day.

Assumptions

- **10 patients per day** — one clinician working an ~8-hour telehealth day, or two clinicians sharing the load
- **\$100 AUD per consult** — mid-market against InstantConsult \$79 and Hola Health \$89; premium positioning for rural urgent care
- **300 operating days per year** — 7-day-week with a small buffer for public holidays and downtime
- **Clinician rate \$25 per consult** — 25% of gross to the treating clinician; 75% retained by the practice. Structured this way because clinicians are also founder-shareholders — value flows back via dividends rather than contractor income

Annual P&L · AUD

Gross revenue · 3,000 × \$100	\$300,000
Clinician contractor payments (\$25 × 3,000)	(\$75,000)
Stripe processing (1.75% + 30¢)	(\$6,000)
eRx e-prescribing fees	(\$3,000)
Insurance uplift (malpractice + cyber)	(\$4,000)
Marketing (10% of revenue)	(\$30,000)
Accounting + platform allocation	(\$5,000)
Pre-tax profit	\$177,000
AU corporate tax (25%, base-rate entity)	(\$44,250)
Post-tax profit	\$132,750

Founder take-home. Rachel and you earn ~\$37.5k each in clinician contractor income (evenly splitting 3,000 consults @ \$25). Plus each founder receives ~\$44k dividend from the \$132.75k post-tax profit (equal 33.3% split). Totals: Rachel ~\$82k, you ~\$82k, Patrick ~\$44k. Scale sensitivity: 15 patients/day → ~\$210k post-tax profit; 20 patients/day → ~\$285k post-tax.

Three to four months to first AU consult.

WEEKS	MILESTONE	OWNER
1-4	Entity formation (Tere Health Pty Ltd), shareholders agreement, IP assignment	Patrick H + AU lawyer
1-4	AHPRA confirmations for clinical roster (Rachel FACEM via TTMRA, Shively existing). Patrick's AHPRA sits behind launch — not on the critical path.	Rachel + Patrick S
2-6	AU banking, Stripe AU, Xero, GST setup	Patrick H
4-12	Product: au_mode flag, AUD pricing, eScript integration, AU pharmacy directory, 000 emergency, MBA-compliant consent copy	Patrick H (engineering)
4-14	AU clinical protocols, provider recruitment (2-3 AU-licensed contractors)	Patrick S
12-16	Soft launch — 1-2 states, rural-focused	All three

The ask: commit as founding partner. First real time-commitment starts at signing. AHPRA verification and initial protocol review are the first two things we'd need from you.

Rural Australia needs this.

We have the platform. We have the founding team. We have the funding to launch. We're inviting you to build the Australian arm with us as an equal partner.

Suggested next steps

- 30-minute call to walk through questions
- Reference conversation with Dr Rachel Thomas (NZ Medical Director, FACEM) if useful
- Term sheet within 2 weeks of your yes
- Independent legal review before signing
- Sign and start ~4 weeks from today